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February 18, 2026

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

Re: Response to Request for Information — Accelerating the Adoption and Use of Artificial Intelligence in Healthcare (Federal Register Doc. 2025-23641)

Dear Secretary Kennedy,

Pursuant to the Department of Health and Human Services' Request for Information titled *"Accelerating the Adoption and Use of Artificial Intelligence as Part of Healthcare Transformation"* (Federal Register, December 23, 2025) and the related HHS public notice, we respectfully submit the enclosed materials for your review and consideration. The State of Alabama is currently evaluating Senate Bill 97 (SB97), legislation that would authorize the creation of a state-guided artificial intelligence healthcare framework known as Health Command. Additionally, the Commonwealth of Kentucky has introduced similar legislation (SB175) modeled upon the same principles of rural healthcare preservation and AI-augmented care delivery. Copies of both legislative instruments are enclosed for your review.

Health Command is designed as a public-facing, state-aligned AI intermediary platform that supports rural hospitals, clinics, health departments, and licensed providers by:

- Augmenting healthcare workforce capacity during critical provider shortages
- Improving triage, preventive care access, and care navigation
- Escalating patients to licensed in-state providers and clinicians
- Reducing unnecessary emergency room utilization
- Strengthening local provider loyalty within state healthcare ecosystems

A central component of the platform is a patented U.S.-developed methodology known as Avatar Tethering, secured under appeal before the U.S. Patent and Trademark Office. This technology enables contextual, ethical interaction between patients, providers, healthcare institutions, and approved sponsors within a remote AI-supported care environment. By tethering relevant care resources and partnerships to individual patient needs, the system promotes faster resolution, coordinated care, and enhanced engagement while preserving human clinical oversight.

Importantly, this framework does not replace licensed medical professionals. Rather, it augments nurses, physicians, and hospital staff at a time when aging populations and shrinking workforces demand scalable support systems. The approach is consistent with HHS goals of improving access, lowering costs, reducing wasteful utilization, and responsibly integrating AI into healthcare delivery.

The enclosed materials include:

- ACoverLetterandOverview.pdf
- AvatarTetheringFramework.pdf
- AvatarTetheringMethodology.pdf
- BillAlabamaSB97.pdf
- BillKentuckySB175.pdf
- Presentation1.pdf
- Presentation2.pdf
- State_Avatar_Alabama.jpg
- State_Avatar_Kentucky.jpg
- TetheringRevenueModel.jpg
- U.S.HEALTHCOMMAND.jpg
- U.S.TetheringRemoteGovernment.jpg

We respectfully submit these materials for consideration as HHS evaluates state-led AI healthcare initiatives under the RFI. We believe this model demonstrates how states can responsibly deploy AI in partnership with federal guidance, particularly within the framework of the Rural Health Transformation (RHT) initiative and related federal priorities.

Should the Department determine it appropriate, we would welcome the opportunity to discuss how Health Command and the Avatar Tethering framework might serve as a pilot or reference model for broader national implementation in partnership with HHS and other federal healthcare entities.

Thank you for your leadership in encouraging states to innovate responsibly in the AI era. We appreciate your consideration and stand ready to provide any additional information the Department may require.

Most Respectfully,

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HHS Request for Information
Accelerating the Adoption and Use of Artificial Intelligence in Clinical Care
Federal Register Doc. 2025-23641
FastHealth Corporation
February 18, 2025

Executive Summary

This submission responds to the Department of Health and Human Services' Request for Information regarding regulation, reimbursement, and research & development priorities for artificial intelligence (AI) in clinical care.

We respectfully present the **Health Command intermediary AI framework**, currently under legislative consideration in Alabama (SB97) and Kentucky (SB175), as a scalable, state-guided model that aligns with federal objectives to:

- Preserve rural healthcare access and sustainability
- Augment workforce capacity
- Reduce wasteful utilization
- Improve preventive and chronic care management via ai remote care
- Enable sustainable, value-aligned reimbursement innovation

Health Command is designed as a public-facing AI care navigation and workforce augmentation system that assists healthcare institutions by providing immediate interaction for remote patients. Ai triage is facilitated through Ai agents optimizing best care by evaluating patient needs. Ai agents presented as Avatars can evaluate patient priorities then escalate patients towards traditional scheduling through in person visits to local providers, or the Ai can also assist to escalate immediate care through a local or state-based telehealth provider based locally. Smaller hospitals can be preserved with Ai agents assisting patients. Preserving human oversight by augmenting providers with Ai assistants helps states compete against reductions in heathcare worker availability. A patented methodology known as **Avatar Tethering** enables contextual care coordination and sustainable revenue support mechanisms within ethical boundaries. Ai intermediaries are branded with local or rural healthcare institutions and provide an ai sanctioned environment where traditional health institutions merge with Ai innovation. This system Transforms Rural Healthcare and prepares for urban healthcare transformation through Ai health agents and remote access and immediate care.

We believe this framework may serve as a pilot-ready starting point for national implementation in partnership with HHS as U.S. Health Command.

Comments Regarding HHS Inclusion

I. Regulation Aspects Related to A Pilot National Health Command Program
A Predictable and Proportionate Regulatory Posture
Health Command is structured as a **non-medical device AI intermediary**, operating as a hybrid innovative provider of healthcare ai partnered with traditional medicine. The AI performs triage, education, navigation, and workflow support functions for established rural or local institutions. This means standards can follow traditional requirements for patient care and responsibility even when Ai assisted established providers and institutions are currently not available.

Final clinical diagnosis, prescribing, and treatment decisions remain under licensed clinicians but doctors can rely on Ai to help in evaluation and prioritization through augmentation and workforce assistance. This system could help HHS undertake initial health institution interaction that leads to in quickly scheduled onsite visits or faster telehealth escalations as needed.

1. **Clarification of AI Intermediary Status**
   HHS could clarify regulatory treatment of AI systems functioning as care navigation and workflow augmentation tools rather than autonomous diagnostic devices.
2. **Risk-Based Regulatory Framework Could Be Determined**
   Regulation could scale proportionally to risk level, distinguishing between:
   - Informational AI tools
   - Workflow augmentation tools
   - Clinical decision support tools
   - Autonomous treatment systems
3. **Risk-Based Regulatory Framework**
   Regulation could scale proportionally to risk level, distinguishing between:
   - Informational AI tools
   - Workflow augmentation tools
   - Clinical decision support tools
   - Autonomous treatment systems
4. **Safe Harbor for State-Guided AI Systems**
   Where AI systems operate under state governance and licensed clinician oversight, HHS could establish safe harbor pathways to encourage innovation while protecting patients.

Health Command aligns with a governance-first model that preserves accountability and public trust.

II. Reimbursement Topics

Modernizing Incentives with Ai Tools to Promote High-Value AI

Legacy fee-for-service payment structures often discourage preventive care and rapid innovation adoption. Health Command supports payment modernization by:

- Reducing unnecessary emergency room visits
- Improving early triage and chronic disease management
- Escalating to high-value, in-network providers
- Supporting Tele health routing within state-based systems

We respectfully recommend HHS consider:

1. **Reimbursement Codes for AI-Augmented Triage and Navigation**
   Establish reimbursement pathways for AI-supported clinical workflows where measurable cost reductions occur.
2. **Shared Savings Models**
   Allow states piloting AI intermediary systems to participate in shared savings agreements tied to reduced ER utilization and improved quality metrics.
3. **Value-Based Demonstration Projects**
   Enable Health Command-style systems to participate in CMMI demonstration projects to test alternative payment frameworks.

The Avatar Tethering methodology further allows contextual educational and sponsorship partnerships to create modest but scalable revenue support for rural institutions without increasing patient financial burden.

III. Research & Development

Translating AI from Concept to Clinical Impact

Health Command presents opportunities for structured public-private collaboration under:

- Cooperative Research and Development Agreements (CRADAs)
- Applied AI implementation research
- Rural care delivery demonstration pilots

HHS could invest in:

1. **Intermediary AI Evaluation Research**
   Studying triage accuracy, workflow efficiency gains, and cost reductions.

2. **Implementation Science**
   Measuring adoption rates, provider acceptance, and patient trust.

3. **Interoperability Pilots**
   Integrating AI intermediary systems with existing EHR platforms using standardized data exchange protocols (FHIR-based systems).

Health Command provides a structured environment to evaluate AI under real-world rural conditions.

IV. Specific Solutions Health Command Presents To Below. Topics or Questions

1. Barriers to Ai Innovation Focus on the Below:

- Regulatory ambiguity
- Misaligned payment incentives
- Liability uncertainty
- Interoperability fragmentation
- Provider adoption resistance

Health Command mitigates these barriers through state governance and provider integration.

2. Health Command Could Evolve Regulatory & Payment Changes

- Clarify AI intermediary classification
- Create reimbursement pathways for AI-supported triage
- Encourage CMMI pilot participation

3. Health Command Could Organize and Solve Current Ai Legal & Governance Issues

- Liability clarity for AI-as-assistant models
- Data privacy protections
- Transparent oversight structures

HHS may provide model governance frameworks through this program.

4. Health Command Could Help Evaluation Methods

- Pre-deployment validation testing
- Post-deployment performance monitoring
- Human-centered workflow evaluation

HHS support via grants or cooperative agreements would accelerate evidence generation.

5. Health Command Could Assist Accreditation & Certification
HHS could support industry-driven certification for AI clinical intermediaries to promote trust and competition. Health Commands platform could expedite adoption of Ai and this could speed up this process for Ai accreditation.

6. Performance Expectations for Augmented Ai
AI tools have shown promise in triage, chronic disease monitoring, and documentation automation. Shortcomings often stem from integration failures rather than technical limits.

7. Adoption Influencers Can Receive Tremendous Transformation from Health Command

- Hospital CEOs
- Chief Medical Officers
- Compliance officers
- IT leadership

Administrative burden and risk uncertainty remain hurdles but a consistent national or state platform aligns professionals with a standard.

8. Interoperability Is Assured with Consistent Systemic Platforms Shared by Many

- Standardized FHIR APIs
- Structured clinical datasets
- Quality benchmarking tools

9. Patient Concerns
Patients seek:

- Faster access
- Reduced travel
- Lower cost

Healthcare Industry Concerns include:

- Data privacy
- Replacement of human care
- Algorithmic bias

Health Command preserves licensed provider oversight to address these concerns while expanding knowledge, education, and prevention.

10. Research Priorities from Health Command Implementation

- AI workforce augmentation becomes real
- Rural hospital sustainability through Ai adaption and new revenue streams
- Preventive care AI provides will increase outcomes
- Cost-reduction modeling with Ai tools gives hope to struggling hospitals

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Why Health Command Represents a National Starting Point
Health Command offers:

- State-guided governance
- Licensed provider oversight
- Workforce augmentation focus
- Rural preservation strategy
- Payment modernization alignment
- Patent-protected infrastructure enabling sustainable support

Rather than fragmented adoption across independent systems, Health Command presents a coordinated, scalable framework that HHS could evaluate as a structured national model.
It preserves human clinicians, strengthens rural institutions, aligns with RHT objectives, and provides measurable opportunities for cost reduction and quality improvement.

Conclusion
We respectfully submit that Health Command aligns directly with HHS's stated goals regarding regulation, reimbursement modernization, and AI-driven research advancement. Alabama and Kentucky have introduced legislation regarding the Avatar Tethering system.

U.S. Health Command could swiftly be implemented through this standardization model deck we are forwarding as it is combining Ai health agents working with providers and patients. Generating targeted partnership revenue for struggling hospitals opens up new revenue opportunities related to the tethering of pharmaceutical advertising avatars presented during Ai agent interaction with patients and telehealth escalation. Topic focused health discussions between ai and patients provide instant promotional value to pharmaceutical sponsorships that could generate billions for rural hospitals, rural clinics, and health departments needs. By offering the convenience of instant Ai interaction through avatars the prevention benefits of remote instant care will increase positive health outcomes during remote care.

FastHealth.AI is a language model developer. It provides admin areas that allow rural hospitals to train Ai with simple text prompts. This allows administration of Ai to be undertaken by non-technical staff at rural hospitals, small clinics, and health departments.

The below are front facing Ai's being used as pilot Ai sites.

https://ai.fasthealth.com/getpage.php?name=DMC (AL)
Dale Medical Center - RURAL HOSPITAL AI

https://ai.fasthealth.com/getpage.php?name=MMHD (CALIFORNIA)
Mayers Memorial Hospital – RURAL HOSPITAL AI

https://ai.fasthealth.com/getpage.php?name=CFP (MISSOURI)
Choices for People – RURAL MENTAL HEALTH AI